5660 W Flamingo RdSTE A
Las Vegas NV 89103
Phone: (702)416-0611 Fax: (702)899-4479
NPI: 1720712235

STAFF INFO
Name: Garcia Alfonso, Kelvyn RN
Patient INFO
Name: MARSHALL, JEFFREY
DOB: 1957-10-21

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
297328

(M0014) Branch State
Branch State **Nevada**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1881327641

Physician
SOLOMON, AUSTIN

Adddress
526 S 24TH ST PHILADELPHIA PA 19146

☐ **Not Applicable**

(M0020) Patient ID number
123962

(M0030) Start of Care Date
03/26/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ **Not Applicable**

(M0040) Patient Name

First Name	Middle Name	Last Name	Suffix
JEFFREY		MARSHALL	

(M0050) Patient State of Residence
State of Residence **Nevada**

(M0060) Patient Zip Code
89115

(M0063) Medicare Number
4F09VR5YC59

☐ **Not Applicable**

(M0064) Social Security Number

☑ **Not Applicable**

(M0065) Medicaid Number

☑ **Not Applicable**

(M0066) Birth Date *
10/21/1957

(M0069) Gender *
Gender **Male**

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ None of the above

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- ☐ None; no charge for current services
- ☑ Medicare (Traditional fee-for-service)
- ☐ Medicare (HMO/managed care/Advantage plan)
- ☐ Medicaid (Traditional fee-for-service)
- ☐ Medicaid (HMO/managed care)
- ☐ Worker's compensation
- ☐ Title programs (for example, Title III, V, or XX)
- ☐ Other government (for example, TriCare, VA)
- ☐ Private insurance
- ☐ Private HMO\managed care
- ☐ Self-pay
- ☐ Other (specify)
- ☐ Unknown

A1110. Language

A.What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0 - No ○ 1 - Yes ○ 9 - Unable to determine

(M0080) Discipline Completing Assessment
◉ RN ○ PT ○ SLP/ST ○ OT

Assigned Staff
Garcia Alfonso, Kelvyn RN

(M0090) Date Assessment Completed:
03/26/2026

(M0100) Assessment Reason

- [x] Start of Care

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)

If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered

mm/dd/yyyy

(M0104) Date of Referral:

Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA

03/24/2026

- [x] NA - No specific SOC/ROC date ordered by physician

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [ ] A - Yes, it has kept me from medical appointments or from getting my medications
- [x] B - Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [ ] C - No
- [ ] X - Patient unable to respond
- [ ] Y - Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1 - Long-term nursing facility (NF)
- [ ] 2 - Skilled nursing facility (SNF/TCU)
- [ ] 3 - Short-stay acute hospital (IPPS)
- [ ] 4 - Long-term care hospital (LTCH)
- [ ] 5 - Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6 - Psychiatric hospital or unit
- [ ] 7 - Other (specify)
- [x] NA - Patient was not discharged from an inpatient facility _[Go to M1021]

(M1005) Inpatient Discharge Date (most recent)

mm/dd/yyyy

- [ ] UK - Unknown

Primary Reason for Home Health / Medical Necessity

The patient is considered homebound because he is recovering from a left partial 5th ray amputation due to gangrene. He presents with a postoperative wound dehiscence on the distal aspect of the incision, which is a full-thickness, non-healing diabetic foot wound requiring negative pressure wound therapy (wound vac) with changes three times a week. Due to this complex wound, underlying Type 2 diabetes mellitus with diabetic peripheral angiopathy, and impaired sensation in the left lower extremity, leaving the home requires a considerable and taxing effort. The patient is instructed to continue weight-bearing as tolerated in a CAM boot/post-op shoe, and any ambulation outside the home carries a significant risk of wound trauma, infection, or disruption, thus impeding the healing process. Absences are limited to medically necessary appointments, such as for wound care. These conditions demonstrate that he meets Medicare criteria for homebound status and justify the need for skilled home health services.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [x] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [ ] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [x] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Amputation.

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Weakness and pain from recent surgery
- [x] Experiencing pain impairs/restricts mobility
- [ ] Visually impaired-requires constant attendance/assist
- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Cognitively impaired, unable to safely leave home without assist
- [x] Leaving home is exhausting, extended rest periods to recover
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Extended respiratory recovery after exertion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Diabetic peripheral neuropathy non-treated with opioids.

Prognosis

- ( ) 1-Poor
- ( ) 2-Guarded
- (x) 3-Fair
- ( ) 4-Good
- ( ) 5-Excellent

Functional Limitations

- (x) 1 - Amputation
- ( ) 2 - Bowel/Bladder (Incontinence)
- ( ) 3 - Contracture
- ( ) B- Other (Specify)
- ( ) 4 - Hearing
- ( ) 5 - Paralysis
- (x) 6 - Endurance
- (x) 7 - Ambulation
- ( ) 8 - Speech
- ( ) 9 - Legally blind
- ( ) A - Dyspnea with minimal exertion

Emergency Preparedness

Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
State and local emergency preparedness officials notified about the possible need for evacuation
Discussed patient specific emergency planning options
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level

Low Risk or Low Priority

Emergency Triage

Category 1 - Life threatening (or potential) and requires ongoing medical treatment.

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments

Advance Directives

Patient has Advance Directives? ( ) Yes (x) No

If you have checked "yes", fill the following fields:

Advance Directives (select the entities that apply to you): ( ) Do Not Resuscitate (DNR) ( ) Living Will

- [ ] Medical Power of Attorney

Name **Phone**

Other

Copies on file at agency? ( ) Yes (x) No

Has surrogate? ( ) Yes (x) No

Name **Phone**

Patient was provided verbal and written information on Advance Directives? (x) Yes ( ) No

Is the Patient DNR (Do not Resuscitate)? ( ) Yes (x) No

Cultural

Primary Language?

- [x] English
- [ ] Spanish
- [ ] Chinese
- [ ] Vietnamese

Other

Is religion important to the patient? ( ) Yes ( ) No

Patient's religious preference:
Patient declined to respond.
Patient's primary source of emotional support:
Relatives.

VITAL SIGNS

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

a. Height (in inches). Record most recent height measure since the most recent SOC/ROC
70

b. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.
200

BMI
28.69

Comments

Vital Signs

Temperature: 97.6 Temporal
Respiratory (per minute): 12
Blood Pressure mm/Hg (R): /
Blood Sugar (mg/dl): 402 Non-Fasting
O2 Sat: 99

Pulse (per minute): Apical = Radial = 71
Weight: 200 reported
Blood Pressure mm/Hg (L): 130 / 78 sitting

O2 Sat in: Room Air

NEUROLOGICAL (HEAD)

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- [x] **0. Never**
- [ ] **1. Rarely**
- [ ] **2. Sometimes**
- [ ] **3. Often**
- [ ] **4. Always**
- [ ] **7. Patient declines to respond**
- [ ] **8. Patient unable to respond**

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year

- ◯ **0. Missed by > 5 years or no answer**
- ◯ **1. Missed by 2-5 years**
- ◯ **2. Missed by 1 year**
- [x] **3. Correct**
- ◯ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month

- ◯ **0. Missed by > 1 month or no answer**
- ◯ **1. Missed by 6 days to 1 month**
- [x] **2. Accurate within 5 days**
- ◯ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week

- ◯ **0. Incorrect or no answer**
- [x] **1. Correct**
- ◯ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("something to wear")**
- [x] **2. Yes, no cue required**
- ◯ **Not assessed/no information**

B) Able to recall "blue"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a color")**
- [x] **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C) Able to recall "bed"

- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a piece of furniture")**
- [x] **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **15**

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?

- [x] **0. No**
- ◯ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- [x] **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- [x] **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- vigilant – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- stuporous – very difficult to arouse and keep aroused for the interview
- comatose – could not be arouse

- [x] **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [x] **Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.**
- ◯ **Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.**
- ◯ **Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility**
- ◯ **Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.**
- ◯ **Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.**

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [x] **Never**
- ◯ **In new or complex situations only**
- ◯ **On awakening or at night only**
- ◯ **During the day and evening, but not constantly**
- ◯ **Constantly**
- ◯ **Patient nonresponsive**

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [x] **None of the time**
- ◯ **Less often than daily**
- ◯ **Daily, but not constantly**
- ◯ **All of the time**
- ◯ **Patient nonresponsive**

Neuro/Emotional/Behavioral Assessment

Conditions of Participation

Psychosocial

- [ ] **WNL (Within Normal Limits)**
- [ ] **Poor Home Environment**
- [ ] **Impaired Decision Making**
- [x] **Poor Coping Skills**
- [ ] **Demonstrated/Expressed Anxiety**
- [ ] **Document as Clinical Narrative**
- [ ] **Agitated**
- [ ] **Inappropriate Behavior**
- [ ] **Depressed Mood**
- [ ] **Irritability**

Educational level

High school.

Primary Language

English.

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

◯ **Needs interpreter**

Learning

◯ **Mental** ◯ **Psychosocial**
● **Physical** ◯ **Functional**

Emotions Reported:

◯ **Angry**
◯ **Discouraged**
◯ **Withdrawn**

◯ **Flat affect**
● **Difficulty coping**
◯ **Disorganized**

◯ **Depressed** ◯ **Recent** ◯ **Long Term**

Treatment:

Evidence of abuse/neglect/exploitation:

◯ **Potential**
◯ **Verbal/Emotional**
◯ **Physical**

◯ **Actual**
◯ **Financial**
◯ **No Evidence of Abuse/Neglect/Exploitation**

Unable to cope with altered health status, patient:

◯ **Lacks motivation** ◯ **Has unrealistic expectations** ◯ **Denies problems** ● **Unable to recognize problems**

Sleep/Rest

● **Adequate** ◯ **Inadequate**

No. of hours slept per night: **7**

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☑ Yes ☐ No ☐ NA

Did the patient have a job before admission? ◯ Yes ◉ No

If yes, do they want to drive post-discharge? ☑ Yes ☐ No

If yes, do they want to return to work post-discharge? ◯ Yes ◯ No

Mental Status:

● **1 - Oriented**
◯ **2 - Comatose**
◯ **3 - Forgetful**
◯ **4 - Depressed**
◯ **5 - Disoriented**
◯ **6 - Lethargic**
◯ **7 - Agitated**
● **8 - Alert**
◯ **9 - Confused**
◯ **10 - Dementia**
◯ **11 - Assaultive**
◯ **12 - Delirious**
◯ **13 - Wanders**
14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert and oriented, patient dress is clean and appropriate cooperative with assessment, express feeling depressed but denies suicidal/homicidal ideation.

Has there been a sudden/acute change in their mental status? ☐ Yes ☐ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Neuromuscular

Impairment:

- [ ] No Problems identified
- [ ] Speech
- [ ] Hearing
- [ ] Visual
- [ ] Tremors
- [x] Numbness
- [x] Tingling
- [ ] Vertigo
- [ ] Ataxia
- [ ] Decreased Sensitivity

Fall(s) since last visit (Specify):

Grip Strength:

- [x] Normal
- [ ] Strong
- [ ] Weak
- [x] Equal
- [ ] Unequal
- [ ] Bilateral
- [ ] Right
- [ ] Left

Others (Specify):

Grasp Strength:

- [x] Normal
- [ ] Strong
- [ ] Weak
- [x] Equal
- [ ] Unequal

Mobility:

- [ ] Balance Within Normal Limits
- [x] Unsteady Gait

Weakness (Specify):
Generalized weakness

Change in ADL (Explain):

EYES AND EARS

B0200. Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- (●) 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- ( ) 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- ( ) 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- ( ) 3. Highly impaired – absence of useful hearing

B1000. Vision

Ability to see in adequate light (with glasses or other visual appliances)

- ( ) 0. Adequate – sees fine detail, such as regular print in newspapers/books
- (●) 1.Impaired – sees large print, but not regular print in newspapers/books
- ( ) 2.Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- ( ) 3.Highly impaired – object identification in question, but eyes appear to follow objects
- ( ) 4.Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Pupils

- [ ] No Problems Identified
- [x] PERRLA
- [ ] Anisocoria
- [ ] Right
- [ ] Left
- [ ] Others(Specify):

Vision

- [ ] No Problems Identified
- [x] Glasses
- [ ] Macular Degeneration
- [ ] Blurred Vision
- [ ] Glaucoma
- [ ] Contact Lenses:
- [ ] Right
- [ ] Left
- [x] Cataracts
- [ ] Blindness(Specify):
- [x] Others (Specify):

Possible macular degeneration due to diabetic retinopathy.

Hearing

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Speech Impairment (Specify):

CARDIOVASCULAR

Heart Sounds

- [x] Normal
- [x] Regular
- [ ] Irregular
- [ ] Murmur
- [ ] Gallops
- [ ] Clicks
- [ ] Friction Rule

Comments (If Abnormal (Explain)):

Edema

Upper:

- [ ] Right Extremities
- [ ] Left Extremities

Lower:

- [ ] Right Extremities
- [ ] Left Extremities

Others:

Comments:
No edema was noted.

.

- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue

Capillary Refill:

- [ ] > 3 secs
- [x] < 3 secs

PT/INR:

/

Peripheral Pulses

Radial:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent
Pedal:	[ ] Strong/Bounding	[x] Weak	[ ] Thready	[ ] Absent

Chest Pain

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Others (Specify):

Associated with:

- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Frequency/Duration:

Comments:

RESPIRATORY (CHEST/THORAX)

M1400. When is the patient dyspneic or noticeably Short of Breath?

- ( ) 0. Patient is not short of breath
- (✓) 1. When walking more than 20 feet, climbing stairs
- ( ) 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- ( ) 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- ( ) 4. At rest (during day or night)

Respiratory Status

- [x] Dyspnea:
- [ ] At Rest
- [x] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor

O2 lpm O2 Saturation 99 % [ ] PRN [ ] Continuous

[ ] Mechanical Ventilator: [ ] Intubated [ ] Tracheostomy Others (Specify):

Lung Sounds

Anterior: Right: Clear Left: Clear
Posterior: Upper Right: Clear Upper Left: Clear Lower Right: Clear Lower Left: Clear

Cough

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic
- [ ] Non-Productive
- [ ] Productive: [ ] Not Assessed/No Information [ ] Yes [ ] No
- Suction: [ ] Not Assessed/No Information [ ] Yes [ ] No
- [ ] Hemoptysis Frequency: Amount:

GASTROINTESTINAL (MOUTH TO ANUS)

K0520. Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[✓]
Z. None of the above	[ ]

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- ( ) Able to independently feed self.
- (✓) Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ( ) Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ( ) Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ( ) Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ( ) Unable to take in nutrients orally or by tube feeding.

(M1620) Bowel Incontinence Frequency

- (✓) Very rarely or never has bowel incontinence
- ( ) Less than once weekly
- ( ) One to three times weekly
- ( ) Four to six times weekly
- ( ) On a daily basis
- ( ) More often than once daily
- ( ) Patient has ostomy for bowel elimination
- ( ) Unknown

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- (✓) Patient does not have an ostomy for bowel elimination
- ( ) The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.
- ( ) Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.

Mouth and Throat

- [x] No Problems Identified
- [ ] Dentures: [ ] Upper [ ] Lower [ ] Partial
- [ ] Bleeding Gums/Gingivitis [ ] Toothache [ ] Ulcerations [ ] Lesion
- [ ] Dysphagia [ ] Hoarseness [ ] Sore Throat [ ] Masses/Tumors

Comments:

Appetite

- [x] Good
- [ ] Fair
- [ ] Poor
- Intake: [ ] Adequate [ ] Inadequate

Abnormalities:

- [ ] Anoxeria
- [ ] Cachexia
- [ ] Nausea
- [ ] Vomiting
- [ ] Decreased Appetite
- [ ] Reflux/Indigestion
- [ ] Diarrhea
- [ ] Constipation
- [ ] Bowel Incontinence
- [ ] Other (Specify):

Feeding

- [ ] Formula
- [ ] Continuous Feeding cc hours
- [ ] Bolus:
- [ ] Gravity
- [ ] Pump cc every hour(s)

Flushing Protocol:

PEG/G-Tube Placement:

- [ ] Placement Checked
- [ ] Residual Checked Amount:

#16 Nutritional Requirements

- [ ] NPO
- [ ] No Added Salt
- [x] Low Salt
- [x] Low Fat
- [x] Low Cholesterol
- [x] No Concentrated Sweets

☐ Controlled Carbohydrates ☐ Regular ☐ Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
○ Without reason, has lost more than 10 lbs in the last 3 months	15	○ Does not always have enough money to buy foods need	10
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	○ Eats alone most of the time	5
○ Has open decubitus, ulcer, burn or wound	10		
○ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
○ Has a tooth/mouth problem that makes it hard to eat	10	○ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
○ Has 3 or more drinks of beer, liquor or wine almost every day	10	○ Frequently has a diarrhea or constipation	5
		Total	15

● Good Nutritional Status (Score 0 - 25) ○ Moderate Nutritional Status (Score 25 - 54) ○ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:

Weight

☑ No Change ☐ Weight Gain ☐ Weight Loss

Abdomen

☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm)

Ostomy/Stoma Appearance

☐ Pinkished-Red ☐ Moist Abnormalities: ☐ Purple ☐ Black in Color ☐ Dry

☐ Gray ☐ Bad Odor ☐ Itches/Hurt Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

☑ Right Lower Normoactive ☑ Right Upper Normoactive ☑ Left Lower Normoactive ☑ Left Upper Normoactive

Last Bowel Movement: 03/25/2026

Constipation

☐ Chronic ☐ Acute ☐ Occasional ☐ Internal Hemorrhoids

☐ External Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance

☑ Within Normal Limits ☐ Gray ☐ Tarry ☐ Black ☐ Fresh Blood

☐ Others (Specify):

GENITOURINARY (GENITALS AND URINARY)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

● No ○ Yes ○ Patient on prophylactic treatment ○ Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

○ Patient is incontinent
- ○ Day time
- ○ Night time
- ○ Day and Night
- ○ Timed-voiding defers incontinence
- ○ Occasional stress incontinence

○ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

● No incontinence or catheter (includes anuria or ostomy for urinary drainage)

Genitourinary Assessment

☑ Within Normal Limits

Abnormalities:

☐ Frequency ☐ Dysuria ☐ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria

☐ Retention ☐ Hematuria ☐ Irrigation ☐ Incontinence ☐ Diapers Used ☐ Penile Discharge

☐ Vaginal Bleeding/Discharge Others (Specify):

Urine Color:

☑ Clear ☐ Transparent ☐ Pale Yellow ☐ Brownish Orange

☐ Pinkish Red ☑ Dark Yellow ☐ Blue or Green ☐ Foamy

☐ Transparent Yellow

Urine Consistency:

☐ Hematuria ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

Last Catheter Change: **Fr:** **cc:**

mm/dd/yyyy

MUSCULOSKELETAL

GG0100. Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.
CODING:

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓	○	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	✓	○	○	○	○
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	✓	○	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- ☐ **A. Manual wheelchair**
- ☐ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☑ **Z. None of the above**

GG0170 - MOBILITY

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0170) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ● **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ● **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.
SOC/ROC Performance

- ○ **(06) Independent**
- ● **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.
SOC/ROC Performance

- ○ **(06) Independent**
- ● **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).
SOC/ROC Performance

- ○ **(06) Independent**
- ● **(05) Setup or clean-up assistance**
- ○ **(04) Supervision or touching assistance**
- ○ **(03) Partial/moderate assistance**
- ○ **(02) Substantial/maximal assistance**
- ○ **(01) Dependent**
- ○ **(07) Patient refused**
- ○ **(09) Not applicable**
- ○ **(10) Not attempted due to environmental limitations**
- ○ **(88) Not attempted due to medical conditions or safety concerns**
- ○ **(-) Not assessed/no information**

(GG0170) F. Toilet transfer: The ability to get on and off a toilet or commode.
SOC/ROC Performance

- ○ (06) Independent
- ● (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.
SOC/ROC Performance

- ○ (06) Independent
- ● (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*
SOC/ROC Performance

- ○ (06) Independent
- ● (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ● (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) O. 12 steps: The ability to go up and down 12 steps with or without a rail.
SOC/ROC Performance

- ○ (06) Independent
- ○ (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.
SOC/ROC Performance

- ○ (06) Independent
- ● (05) Setup or clean-up assistance
- ○ (04) Supervision or touching assistance
- ○ (03) Partial/moderate assistance
- ○ (02) Substantial/maximal assistance
- ○ (01) Dependent
- ○ (07) Patient refused
- ○ (09) Not applicable
- ○ (10) Not attempted due to environmental limitations
- ○ (88) Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170) Q. Does patient use a wheelchair and/or scooter?

- ● (0) No **[GOTO M1600]**
- ○ (1) Yes
- ○ Not assessed (no information)

(GG0170) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.
SOC/ROC Performance

- (06) Independent
- (05) Setup or clean-up assistance
- (04) Supervision or touching assistance
- (03) Partial/moderate assistance
- (02) Substantial/maximal assistance
- (01) Dependent
- (07) Patient refused
- (09) Not applicable
- (10) Not attempted due to environmental limitations
- (88) Not attempted due to medical conditions or safety concerns
- (-) Not assessed/no information

(GG0170) RR1. Indicate the type of wheelchair or scooter used.

- (1) Manual
- (2) Motorized
- Not assessed (no information)

(GG0170) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- (06) Independent
- (05) Setup or clean-up assistance
- (04) Supervision or touching assistance
- (03) Partial/moderate assistance
- (02) Substantial/maximal assistance
- (01) Dependent
- (07) Patient refused
- (09) Not applicable
- (10) Not attempted due to environmental limitations
- (88) Not attempted due to medical conditions or safety concerns
- (-) Not assessed/no information

(GG0170) SS1. Indicate the type of wheelchair or scooter used.

- (1) Manual
- (2) Motorized
- Not assessed (no information)

Gait and Mobility Abnormalities

- [ ] No Problems Identified
- [x] Weakness
- [x] Poor Balance
- [ ] Bed Bound
- [ ] Chair Bound
- [ ] Joint Stiffness
- [ ] Shuffling/Wide-based Gait
- [x] Orthopedic Aftercare
- [x] High Risk for Falls
- [x] Weight Bearing Restriction
- [ ] Poor Conditioning
- [ ] Fracture:
- [ ] Joints Pain:
- [x] Amputation: 5th left toe.
- [ ] Atrophy:
- [ ] Paresthesia:
- [ ] Contractures: Site
- [ ] Foot Drop: [ ] Right [ ] Left

Range of Motion

- [x] Limited Range of Motion: [ ] RUE [ ] RLE [ ] LUE [x] LLE
- [ ] Quadriplegia
- [ ] Hemiplegia
- [ ] Paraplegia

Comments:

INTEGUMENTARY (SKIN)

M1306. Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- 0. No → Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC
- 1. Yes

M1311. Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non¬ removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.

Stage 1 Pressure Injuries **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- No **[GO TO M1340]**
- Yes, patient has BOTH observable and unobservable stasis ulcers
- Yes, patient has observable stasis ulcers ONLY
- Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non¬ removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- One
- Two
- Three
- Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- Fully granulating
- Early/partial granulation
- Not healing

(M1340) Does this patient have a Surgical Wound?

- No **[GO TO N0415]**
- Yes, patient has at least one observable surgical wound
- Surgical wound known but not observable due to non-removable dressing/device **[GO TO N0415]**

(M1342) Status of Most Problematic Surgical Wound that is Observable

- Newly epithelialized
- Early/partial granulation
- Fully granulating
- Not healing

Skin Color

- [ ] Within Normal Limits
- [x] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:

- [ ] Good
- [x] Fair
- [ ] Poor

Skin Abnormalities:

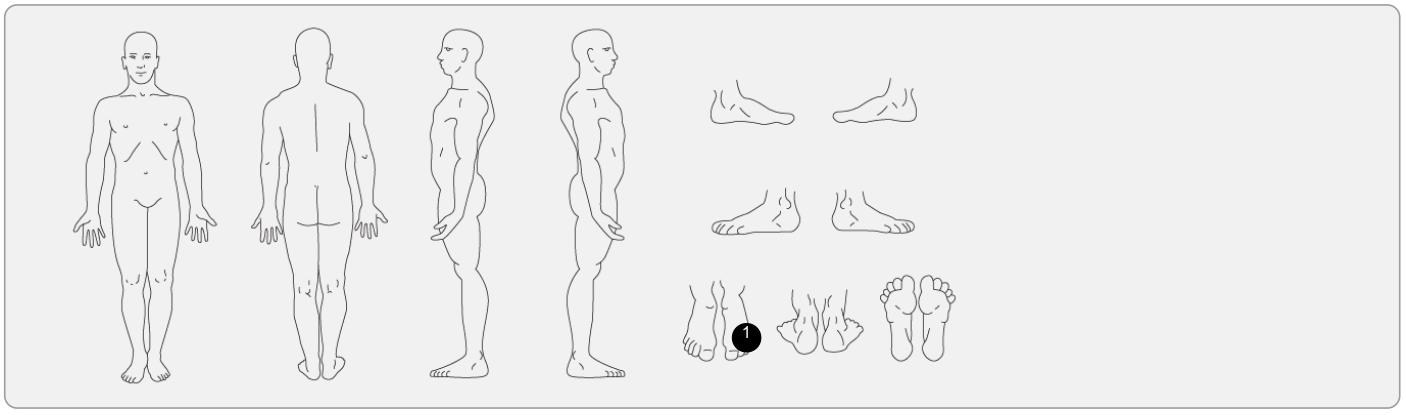
- [x] Dry
- [ ] Warm
- [ ] Cool
- [ ] Rash
- [ ] Ulcer
- [ ] Ostomy
- [x] Incision
- [ ] Diaphoretic
- [ ] Others (Specify):

Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	○ Ambulant	● Walks With Help	○ Chairbound	○ Bedfast
Mobility	○ Full	● Slightly Impaired	○ Very Limited	○ Immobile
Incontinence	● None	○ Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	17

Wound

Fetch Wounds From Previous Wound Note



Wound 1

Location: Left dorsal foot (5th toe amputated).	Size: 5.5 x 2.0 x 0.3	Pressure Injury Stage: -
Type: Surgical	Drainage: Serous	Amount Of Drainage: Minimal (<25% satur)
Odor: None	Color: White	Tunneling/Undermining: No.
Tunneling Location: -	Tunneling Length (cm): -	Undermining Location: -
Undermining Length (cm): -	Surrounding Tissue: Pale	Wound Base: Non-Granulating
Level Of Pain: -	Edema: No.	Stoma: -
Slough and/or Echar: No	Slough (%): -	Eschar (%): -
Granulation (%): -	Pink/Red (%): -	Necrotic (%): -
Wound Status: Non-Healing	Wound Description: -	Satisfaction of Return Demonstration: Yes - Needs Additional Education
Patient's Response to Wound Care: Good - Patient tolerated well.	Wound Care: SN performed hand hygiene, and don gloves. Cleaned the wound using wound cleanser and non-sterile gauze, dried with sterile gauze sponge. Packed with iodoform packing strips. Covered with non-adhesive pad. Wrapped with Kerlix and coban.	
Onset Date: 2026-03-26		

PAIN ASSESSMENT

J0510. Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

- ○ 0. Does not apply – I have not had any pain or hurting in the past 5 days
- ○ 3. Frequently

- [x] 1. Rarely or not at all
- [ ] 2. Occasionally
- [ ] 4. Almost constantly
- [ ] 8. Unable to answer

J0520. Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

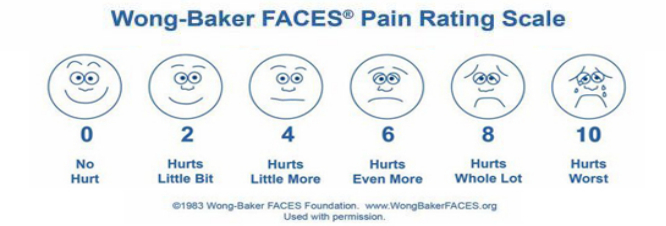
- [x] 0. Does not apply – I have not received rehabilitation therapy in the past 5 days
- [ ] 1. Rarely or not at all
- [ ] 2. Occasionally
- [ ] 3. Frequently
- [ ] 4. Almost constantly
- [ ] 8. Unable to answer

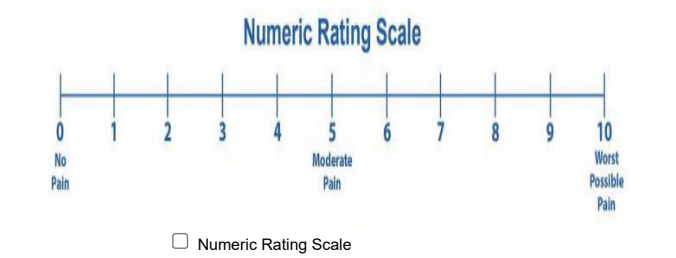
J0530. Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

- [x] 1 - Rarely or not at all
- [ ] 2 - Occasionally
- [ ] 3 - Frequently
- [ ] 4 - Almost Constantly
- [ ] Unable to answer

Pain





3

Collected Using: [ ] Face Scale [ ] Numeric Rating Scale

Pain Location and Description:

1

Location	Start Date	Activity	What makes pain worse?
Bilateral feet.	mm/dd/yyyy	At Rest×	Position×

Intensity	Duration	What makes pain better?	Does pain affect quality of life?
3×		Medication×	Yes.

Description	Frequency	Type of Intervention
Burning×	Frequent×	Non Opiod Analgesic×

Unable to Assess [ ]

Unable to Assess [ ]

How well it is working	Status
Fairly Well×	

EMOTIONAL/MOOD

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	No ×	Never or 1 day
B. Feeling down, depressed, or hopeless	No ×	Never or 1 day
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^

	Presence	Frequency
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

D0700. Social Isolation

How often do you feel lonely or isolated from those around you?

- ○ 0 - Never
- ● 1 - Rarely
- ○ 2 - Sometimes
- ○ 3 - Often
- ○ 4 - Always
- ○ 7 - Patient declines to respond
- ○ 8 - Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- ☐ 1 - Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- ☐ 2 - Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- ☐ 3 - Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- ☐ 4 - Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- ☐ 5 - Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- ☐ 6 - Delusional, hallucinatory, or paranoid behavior
- ☑ 7 - None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- ● 0 - Never
- ○ 1 - Less than once a month
- ○ 2 - Once a month
- ○ 3 - Several times each month
- ○ 4 - Several times a week
- ○ 5 - At least daily

FUNCTIONAL STATUS

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- ○ 0 - Able to groom self unaided, with or without the use of assistive devices or adapted methods.
- ● 1 - Grooming utensils must be placed within reach before able to complete grooming activities.
- ○ 2 - Someone must assist the patient to groom self.
- ○ 3 - Patient depends entirely upon someone else for grooming needs

(M1810)

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- ○ 0 - Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.
- ● 1 - Able to dress upper body without assistance if clothing is laid out or handed to the patient.
- ○ 2 - Someone must help the patient put on upper body clothing.
- ○ 3 - Patient depends entirely upon another person to dress the upper body.

(M1820)

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- ○ 0 - Able to obtain, put on, and remove clothing and shoes without assistance.
- ● 1 - Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.
- ○ 2 - Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.
- ○ 3 - Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- ○ 0 - Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- ● 1 - With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- ○ 2 - Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- ○ 3 - Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ○ 4 - Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ○ 5 - Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- ○ 6 - Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ 0 - Able to get to and from the toilet and transfer independently with or without a device
- ● 1 - When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ○ 2 - Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ○ 3 - Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- ○ 4 - Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ 0 - Able to manage toileting hygiene and clothing management without assistance.
- ● 1 - Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ○ 2 - Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ○ 3 - Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ 0 - Able to independently transfer.
- ● 1 - Able to transfer with minimal human assistance or with use of an assistive device.
- ○ 2 - Able to bear weight and pivot during the transfer process but unable to transfer self.
- ○ 3 - Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ○ 4 - Bedfast, unable to transfer but is able to turn and position self in bed.
- ○ 5 - Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ 0 - Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ● 1 - With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.

- ◯ 2 - Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ◯ 3 - Able to walk only with the supervision or assistance of another person at all times.
- ◯ 4 - Chairfast, unable to ambulate but is able to wheel self independently.
- ◯ 5 - Chairfast, unable to ambulate and is unable to wheel self
- ◯ 6 - Bedfast, unable to ambulate or be up in a chair.

GG0130 - SELF CARE

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.
06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.
If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130)SOC/ROC Performance

A. Eating: The ability to use suitable utensils to bring food and/or liquid to the mouth and swallow food and/or liquid once the meal is placed before the patient.
B. Oral Hygiene: The ability to use suitable items to clean teeth. Dentures (if applicable): The ability to insert and remove dentures into and from mouth, and manage denture soaking and rinsing with use of equipment.
C. Toileting Hygiene: The ability to maintain perineal hygiene, adjust clothes before and after voiding or having a bowel movement. If managing an ostomy, include wiping the opening but not managing equipment.
E. Shower/bathe self: The ability to bathe self, including washing, rinsing, and drying self (excludes washing of back and hair). Does not include transferring in/out of tub/shower.
F. Upper body dressing: The ability to dress and undress above the waist; including fasteners, if applicable.
G. Lower body dressing: The ability to dress and undress below the waist, including fasteners; does not include footwear.
H. Putting on/taking off footwear: The ability to put on and take off socks and shoes or other footwear that is appropriate for safe mobility; including fasteners, if applicable.

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear
(06) Independent	◯	◯	◯	◯	◯	◯	◯
(05) Setup or clean-up assistance	✓	✓	✓	✓	✓	✓	✓
(04) Supervision or touching assistance	◯	◯	◯	◯	◯	◯	◯
(03) Partial/moderate assistance	◯	◯	◯	◯	◯	◯	◯
(02) Substantial/maximal assistance	◯	◯	◯	◯	◯	◯	◯
(01) Dependent	◯	◯	◯	◯	◯	◯	◯
(07) Patient refused	◯	◯	◯	◯	◯	◯	◯
(09) Not applicable	◯	◯	◯	◯	◯	◯	◯
(10) Not attempted due to environmental limitations	◯	◯	◯	◯	◯	◯	◯
(88) Not attempted due to medical conditions or safety concerns	◯	◯	◯	◯	◯	◯	◯
(-) Not assessed/no information	◯	◯	◯	◯	◯	◯	◯

DME and Supplies

DME Lookup

Durable Medical Equipment:
Cane

Wound Supplies:
4x4's, Cotton tipped applicators, Wound cleanser, Non-sterile Gloves, Kerlix

Diabetic Supplies

#18B ACTIVITIES PERMITTED

- ◯ 1- Complete bedrest
- ◯ 2 - Bedrest/BRP
- ✓ 3 - Up as tolerated
- ✓ 4 - Transfer bed/chair
- ✓ 5 - Exercises prescribed
- ✓ 6 - Partial weight bearing
- ◯ 7 - Independent in home
- ◯ 8 - Crutches
- ✓ 9 - Cane

- ◯ A - Wheelchair
- ◯ B - Walker
- ◯ C - No restrictions
- ◯ D - Other (specify)

Assist with ADLs

DIAGNOSIS

M1100. Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯	◯	◯	◯	◯
B. Patient lives with other person(s) in the home	✓	◯	◯	◯	◯
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯	◯	◯	◯	◯

Primary Caregiver	Relationship to Patient	Contact Number
Karen Marshal	Spouse	459-222-299

Comments
Name Of Facility

Able to safely care for patient?

Phone: ☑ Yes ☐ No

Home Environment / Home Safety Assessment

☐ Architectural Barriers	☐ No fire extinguisher	☑ No emergency exit plan
☐ Medications are improperly stored	☐ Inadequate electricity / cooling / heating	☐ Rodent / insect infestation
☐ No telephone	☐ Inadequate supply of water	☐ Unsafe electrical wiring
☑ Cluttered surroundings	☐ Inadequate sanitation / plumbing	☐ Nonworking stove / refrigerator
☐ Inadequate railing	☐ Inadequate smoke detectors on all levels of home	☐ Exposed flammables
☐ Unsafe oxygen use	☐ Unsafe storage of equipments	☐ Inadequate Lighting

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL(Bathing, dressing, toileting, eating/feeding, bowel)	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Psychosocial Support	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Assistance with doctors appointment, delivery of medications	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Management of Finances	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations

Comments:

Safety Hazards Affecting Patient:

☐ Stairs	☐ No running water, plumbing
☐ Inadequate lighting, heating and cooling	☑ Narrow or obstructed walkway
☐ Insect/Rodent Infestation	☐ Lack of fire safety devices
☐ No gas/electric appliance	☑ Cluttered/soiled living area
☐ No hazards identified	

Others:

(M2102) Types and Sources of Assistance:

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ○ **No assistance needed –patient is independent or does not have needs in this area**
- ● **Non-agency caregiver(s) currently provide assistance**
- ○ **Non-agency caregiver(s) need training/ supportive services to provide assistance**
- ○ **Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance**
- ○ **Assistance needed, but no non-agency caregiver(s) available**

Safety Measures

● 1 - Bleeding precautions	● 4 - Fall precautions	○ 7 - Elevate head of bed	○ 10 - Lock w/c with transfers
○ 2 - O_2 precautions	○ 5 - Aspiration precautions	○ 8 - 24 hr. supervision	● 11 - Infection control measures
○ 3 - Seizure precautions	○ 6 - Siderails up	● 9- Clear Pathways up	● 12 - Walker/cane
			● 13 - Sharps Precaution

● 14 - Other:
ER/911 protocol, COVID-19, and universal precautions

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity
PRIMARY DIAGNOSIS ☐ T87.81 - Dehiscence of amputation stump	03/26/2026	○ 0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate
OTHER DIAGNOSIS - 1 E11.52 - Type 2 diabetes w diabetic peripheral angiopathy w gangrene	03/26/2026	○ 0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate
OTHER DIAGNOSIS - 2 L03.032 - Cellulitis of left toe	03/26/2026	

		○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
		○ Onset ◉ Exacerbate
OTHER DIAGNOSIS - 3 Z91.81 - History of falling	**Onset Date** 03/26/2026	**Severity** ○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^ ◉ Onset ○ Exacerbate
OTHER DIAGNOSIS - 4 Z79.4 - Long term (current) use of insulin	**Onset Date** 03/26/2026	**Severity** ○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^ ◉ Onset ○ Exacerbate
OTHER DIAGNOSIS - 5 Z79.84 - Long term (current) use of oral hypoglycemic drugs	**Onset Date** 03/26/2026	**Severity** ○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^ ◉ Onset ○ Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- [x] Diabetes Mellitus (DM)
- [x] Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- [ ] None of the above

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [ ] 1 - History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2 - Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [x] 3 - Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4 - Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5 - Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6 - Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7 - Currently taking 5 or more medications
- [ ] 8 - Currently reports exhaustion
- [ ] 9 - Other risk(s) not listed in 1 - 8
- [ ] 10 - None of the above

Additional Risk Information:

Prior Hospitalization

◉ Yes ○ No

Number of times
3

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- [x] > 1 Hospitalizations or ER visits in the past 12 months
- [x] History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- [ ] HF
- [x] Diabetes
- [ ] COPD
- [ ] Chronic skin ulcers (Wound consult if indicated for any wounds)
- [ ] HIV/AIDS

Risk factors: (Check all that apply)

- [ ] Discharged from hospital or skilled nursing facility
- [x] More than 2 secondary diagnoses
- [ ] Low socioeconomic status or financial concerns ♦
- [ ] Lives alone ►♦
- [ ] Inadequate support network ♦
- [x] ADL assistance needed ►
- [x] Home safety risks ► ♦
- [ ] Dyspnea ► ★
- [ ] Help with managing medications needed ► ★
- [x] Non-compliance with medication regimen ♦ ★
- [ ] Confusion ♦ ★
- [ ] Pressure ulcer ★
- [ ] Stasis ulcer ★
- [ ] Overall Poor Status/Prognosis ■
- [ ] Low literacy level ♦
- [ ] Depression ♦

- [x] ►Consider Therapy referral (PT, OT, ST)
- [x] ♦Consider MSW referral
- [ ] ■ Consider Hospice referral
- [x] ★Consider RN referral, if not ordered

Total number of checked boxes is
9

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- [x] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [ ] Dietary Consultant

Other
Wound care

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- [x] Medication Management
- [x] Medication Reconciliation

- [x] Assess patient's: knowledge, ability, resources and adherence
- [x] Education
- [x] Patient/family education
- [ ] Enrollment into a disease management program

(Specify)

- [ ] Hospice/Palliative Referral
- [ ] Phone Monitoring
- [ ] Front-loading Visits
- [x] Individualized Patient Emergency Care Plan
- [ ] Telemonitoring
- [ ] Care Coordination (physicians, hospitals, nursing homes ...)
- [x] Fall Prevention Program
- [x] Wound Care

Immunizations (M1040, M1045, M1050, M1055)

- [ ] Influenza
- [ ] Pneumococcal

Fall Prevention Program
Discussed fall-preventative measures: slow position changes, asking for help as needed, maintaining adequate lighting, keeping the pathway clear and clutter-free, and using an assistive device for safety.
Other

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	✓	
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level		✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.		✓
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.	✓	
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	✓	
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.	✓	
Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	✓	
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	✓	
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.		✓
A score of 4 or more is considered at risk for falling	7	

MEDICATION AND ALLERGIES

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	■
E. Anticoagulant	☐	■
F. Antibiotic	☐	■
H. Opioid	☐	■
I. Antiplatelet	☑	☑
J. Hypoglycemic (including insulin)	☑	☑
Z. None of the Above	☐	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/26/2026	Amlodipine Besylate	5 mg 1 tablet Once a day	By mouth	Calcium Channel Blockers / HTN	
☐	03/26/2026	Atorvastatin (Oral Pill)	80 mg 1 Tab Once a day	By mouth	ANTILIPEMIC AGENTS /	New
☐	03/26/2026	ASPI-COR	81 mg DR Tab Once a day	By mouth	NON-OPIOID ANALGESICS /	New
☐	03/26/2026	Furosemide (Oral Pill)	40 mg 1 Tab Once a day	By mouth	LOOP DIURETICS /	New
☐	03/26/2026	Metoprolol XR (Oral Pill)	50 mg 24 HR XR Tab Nightly	By mouth	BETA BLOCKERS/RELATED /	New
☐	03/26/2026	Metoprolol XR (Oral Pill)	100 mg 24 HR XR Cap Once a day	By mouth	BETA BLOCKERS/RELATED /	New
☐	03/26/2026	Prasugrel (Oral Pill)	10 mg 1 Tab Once a day	By mouth	PLATELET AGGREGATION INHIBITORS /	New
☐	03/26/2026	Isosorbide Mononitrate (Oral Pill)	30 mg 1 Tab Once a day	By mouth	ANTIANGINALS /	New

	Start Date		Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/26/2026	Ranolazine (Oral Granules)	1,000 mg Granules Once a day	Oral Granules	ANTIANGINALS /	New
☐	03/26/2026	Empagliflozin (Oral Pill)	10 mg 1 Tab Once a day	By mouth	ORAL HYPOGLYCEMIC AGENTS,ORAL / TIIDM	New
☐	03/26/2026	BASAGLAR	100 unt/ml 26 units At bedtime	Subcutaneous	INSULIN / TIIDM	New
☐	03/26/2026	Gabapentin (Oral Pill)	600 mg 1 Tab At bedtime	By mouth	ANTICONVULSANTS /	New
☐	03/26/2026	Magnesium oxide	400 mg 1 tab Once a day	By mouth	Vitamins, others. /	
☐	03/26/2026	Potassium bicarbonate	10 mEq 1 tab Once a day	By mouth	Electrolytes others. /	
☐	03/26/2026	Pantoprazole (Oral tablets)	40 mg 1 tablet Once a day	By mouth	GASTRIC MEDICATIONS,OTHER /	New
☐	03/26/2026	MIRALAX	17 g Pwdr Once a day	Oral Solution Powder	HYPEROSMOTIC LAXATIVES /	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Does patient have IV access? ◯ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ● **No – No issues found during review _[GO TO M2010]**
- ◯ **Yes – Issues found during review**
- ◯ **Not assessed/no information**
- ◯ **NA – Patient is not taking any medications _[GO TO M2102]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ◯ **No**
- ◯ **Yes**
- ◯ **Not assessed/no information**

M2010. Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ◯ **NO**
- ● **YES**
- ◯ **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

M2020. Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ◯ **Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ◯ **Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ◯ **Unable to take medication unless administered by another person**
- ◯ **NA - No oral medications prescribed**

M2030. Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ **Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ◯ **Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ◯ **Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ● **Unable to take injectable medication unless administered by another person**
- ◯ **NA - No injectable medications prescribed.**

Allergies

Name	Reaction	Start Date
		None known

☐ **Add Allergies**

(O0110). Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	☐
A3. Oral	☐
A10.Other	☐
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☐

C2. Continuous	☐
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	☐
D3. As needed	☐
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	☐
G3. CPAP	☐
Others	
H1. IV Medications	☐
H2. Vasoactive medications	☐
H3. Antibiotics	☐
H4. Anticoagulation	☐
H10.Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

PATIENT COORDINATION AND CARE PLAN

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 2/wk x wk4	**MSW Frequency:**	**PT Frequency: Evaluation and Treatment**	**OT Frequency:**
HHA Frequency:	**ST Frequency**	**Dietitian Frequency:**	

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO AVERY HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

REASON FOR HOMEBOUND:
The patient is considered homebound because he is recovering from a left partial 5th ray amputation due to gangrene. He presents with a postoperative wound dehiscence on the distal aspect of the incision, which is a full-thickness, non-healing diabetic foot wound requiring negative pressure wound therapy (wound vac) with changes three times a week. Due to this complex wound, underlying Type 2 diabetes mellitus with diabetic peripheral angiopathy, and impaired sensation in the left lower extremity, leaving the home requires a considerable and taxing effort. The patient is instructed to continue weight-bearing as tolerated in a CAM boot/post-op shoe, and any ambulation outside the home carries a significant risk of wound trauma, infection, or disruption, thus impeding the healing process. Absences are limited to medically necessary appointments, such as for wound care. These conditions demonstrate that he meets Medicare criteria for homebound status and justify the need for skilled home health services.

FREQUENCY AND DURATION:
HH SN frequency: 2 times a week for 4 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: For evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training.

WOUND CARE ORDER:
Surgical Wound on Left dorsal foot (5th toe amputated): Per Dr. Solomon's MA Nataly, basic wound care is fine for now. Clean the wound using wound cleanser and non-sterile gauze, dried with sterile gauze sponge. Packed with iodoform packing strips. Covered with non-adhesive pad. Wrapped with Kerlix and Coban. Change dressing 2-3 days or as needed if soiled or soaked.

GOALS:
-Patient will demonstrate progress in healing of the postoperative wound, with decreased drainage and no signs of infection, as evidenced by nursing assessment and wound vac monitoring within 4 weeks.
-Patient will verbalize understanding of negative pressure wound therapy (wound vac) care, including precautions and signs of malfunction or complications, with caregiver reinforcement as needed within 4 weeks.
-Patient will maintain stable blood glucose levels within target range, through skilled monitoring and teaching of diabetes management strategies within 4 weeks.
-Patient will demonstrate safe ambulation with CAM boot/post-op shoe, requiring fewer verbal cues from nursing staff or caregiver to protect the surgical site within 4 weeks.
-Patient will remain free from falls or wound trauma in the home, as evidenced by nursing documentation and caregiver report within 4 weeks.

Patient Strengths

☑ **Motivated Learner**	☑ **Strong Support System**	☐ **Absence of Multiple Diagnosis**
☐ **Enhanced Socioeconomic Status**	**Other:**	

Conclusions

☑ **Skilled Intervention Needed**	☑ **Skilled Instruction Needed**	☐ **No Skilled Service Needed**

Other:

Patient demographics for Jeffrey Marshall, a 68-year-old male, indicate that he has been referred for home health care on March 24, 2026. His attending physician is Dr. Solomon Austin, and the discipline completing the assessment is nursing, specifically by RN Kelvyn Garcia Alfonso. The start of care date is set for March 26, 2026. It is noteworthy that the patient was not discharged from an inpatient facility within the past 14 days.

Mr. Marshall's homebound status is confirmed due to multiple criteria. He is homebound because of illness and injury, specifically status post-surgery and amputation. Leaving his home is medically contraindicated, as he experiences weakness and pain from recent surgery, which restricts his mobility. Additionally, he has difficulty tolerating standing or walking for extended periods, experiences pain that impairs his movement, and has balance issues that increase his risk for falls. The effort required for him to leave home is considerable and taxing, necessitating the use of assistive devices for ambulation and making activities outside of the home strenuous.

The primary reason for Mr. Marshall's need for home health services is his primary diagnosis of T87.81, which denotes dehiscence of the amputation stump. He also presents with significant comorbidities, including E11.52 type 2 diabetes with diabetic peripheral angiopathy and gangrene, L03.032 cellulitis of the left toe, a history of falling, and long-term use of insulin and oral hypoglycemic medications. These conditions collectively contribute to his overall health challenges and the necessity for skilled home health care.

A thorough head-to-toe assessment revealed that Mr. Marshall's cardiopulmonary status is stable, with a heart rate of 71 beats per minute and a respiratory rate of 12 breaths per minute. His gastrointestinal status appears normal, though he does have a therapeutic diet plan due to his diabetes, which includes low salt, low fat, and low cholesterol recommendations. The integumentary system assessment has identified a non-healing surgical wound, a critical area of concern for ongoing care. His safety assessment highlights the need for continued monitoring due to his impaired balance and mobility.

Psychosocially, Mr. Marshall's status indicates he lives with others in the home and has non-agency caregivers providing assistance. Despite expressing feelings of depression, he denies any suicidal or homicidal ideation. His mental status is assessed via the Brief Interview for Mental Status (BIMS), where he scored a 15, indicating he is oriented and alert. He does not exhibit any signs of cognitive impairment or significant psychiatric symptoms.

Vital signs taken during the assessment show a temperature of 97.6°F, blood pressure of 130/78 mmHg, and a blood sugar level of 402 mg/dL. These readings, particularly the elevated blood sugar, suggest the need for careful management of his diabetes and other health conditions. Mr. Marshall's cognitive function is intact, as he can follow directions and recall tasks independently.

In terms of activities of daily living (ADLs), Mr. Marshall requires setup or clean-up assistance for grooming, dressing, bathing, and toileting. He demonstrates some independence in specific tasks but needs support to access necessary items. His functional ability allows him to ambulate independently using assistive devices, but he still requires help with certain components of his self-care routine.

Mr. Marshall's mobility assessment indicates he can walk on even and uneven surfaces using a cane or crutch but has limitations due to pain and weakness. His ability to transfer is feasible with minimal human assistance or assistive devices, which is crucial for his safety and independence. Social needs within his residence include ensuring he has access to assistance and support from his caregivers.

A detailed review of Mr. Marshall's medications identifies multiple prescriptions, including antiplatelet and hypoglycemic agents, which present a risk for adverse effects and interactions. He is currently taking five or more medications, highlighting the potential for hospitalization if not managed properly. Caregiver education regarding high-risk medications has been provided to Mr. Marshall and his support network.

The primary caregiver for Mr. Marshall is identified as a non-agency family member who assists with daily tasks and health management. Other supports include local community resources that may provide additional assistance during emergencies or health crises. It's essential to continue fostering this support system to ensure Mr. Marshall's needs are met effectively.

In summary, the medical necessity for skilled services in home health care is clearly established based on Mr. Marshall's complex medical conditions, including his recent surgery and ongoing care needs related to diabetes management and wound care. The visit frequency recommended for Mr. Marshall is at least two to three times a week to ensure proper monitoring, intervention, and education regarding his health conditions, medications, and overall well-being. This approach will help to minimize complications and facilitate recovery while maintaining his independence in a home setting.

Skilled Intervention

Assessment/Instruction/Performance:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment on all body systems with OASIS elements evaluation was initiated. Reconciled facility discharges medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed plan of care draft with patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

SN provides comprehensive postoperative care for the patient recovering from amputation, including routine wound assessment, monitoring for signs of infection, and performing dressing changes to promote healing. Pain management is closely supervised, with interventions tailored to support comfort and encourage participation in mobility training. SN reinforces weight-bearing restrictions and supervises safe ambulation with prescribed devices to reduce fall risk and protect the surgical site. SN teaching is directed toward the patient and caregiver, emphasizing the importance of daily inspection of the residual limb, proper hygiene, and adherence to follow-up appointments. SN education also includes recognizing early warning signs of complications such as increased pain, swelling, or drainage, and strategies for safe performance of activities of daily living. Through skilled oversight and structured teaching, the nurse supports wound healing, prevents complications, and promotes the patient's safe adjustment to functional independence within the home environment.

Assessed skin for breakdown and surgical incision on left 5th toe. Instructed patient/caregiver to notify SN/MD for signs/symptoms of wound infection such as fever, chills, and purulent wound drainage with foul odor. Instructed patient/caregiver to keep surgical incision clean, dry, and intact. Not to take tub baths, pools, or spas for 6 weeks (about 1 and a half months) after surgery but you may shower 10 days (about 1 and a half weeks) after surgery. Instructed patient/caregiver on diet high in vitamin, C, E, and protein to promote wound healing. Instructed patient/caregiver on infection control techniques which includes proper handwashing, use of gloves, and proper handling and disposal of waste.

Instructed patient to adhere and take pain medication before pain becomes severe to achieve pain control. Provided teachings on nonpharmacologic pain relief measures such as relaxation techniques and positioning. Instructed patient/PCG to notify SN/MD if patient is experiencing pain level greater than 7/10, unable to tolerate pain, and if pain is affecting ability to perform normal activities/ADLs.

Assessed pt's safety in environment, mobility and functional limitations to reduce falls or accidents every visit. Assessed the patient's ability to perform ADLs effectively and safely every visit. Instructed Patient/Caregiver on the importance of assessing conditions/risks that may contribute to occurrence of falls such as fatigue/ muscle weakness. Encouraged to perform exercises if tolerated for strengthening and maintenance of posture and flexibility. Established HEP and educate Patient/Caregiver on passive/active assistive ROM exercises and encouraged to perform exercises to maintain muscle strength and endurance

Medication reconciliation was done during the visit. Patient/caregiver verbalized understanding in all teachings during the visit.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 60%	☑ **Caregiver (%):** 70%
Return Demonstration	☑ **Patient (%):** 60%	☑ **Caregiver (%):** 70%
Require Further Teaching	☑ **Patient**	☑ **Caregiver**

Comments:
Patient verbalized 60% of understanding on the disease process taught during this visit. Patient unable to teach back complete understanding on fall precautions and hypertension urgency symptoms. Patient also redemonstrated 60% of home safety and fall prevention measures using assistive device walker as patient still demonstrates weakness, poor endurance and ambulation difficulty. CG verbalized 70% of understanding and is available to assist patient for safety with ADLs/IADLs, medication and diet compliance for the patient. CG also redemonstrated 70% of home safety and fall prevention measures using assistive device walker as patient still demonstrates weakness, poor endurance and ambulation difficulty. Both patient and caregiver require further teaching.

Conference With:

☑ **MD**	☑ **SN**	☑ **PT**	☐ **OT**
☐ **ST**	☐ **MSW**	☐ **HHA**	☐ **DON/DOPCS**
☑ **Clinical Coordinator**	☑ **Clinical Manager**		

Name:
Nariah Hinds, RN CM
Regarding:
SOC, POC, pt.'s status and frequency
Physician Contacted Re:
SOC, POC, pt.'s status and frequency
Order Changes:
Plan for Next Visit:
Will continue to assess and evaluate patient understanding, functional status, and knowledge of self-care management on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Initiated education and training provided by the SN, as appropriate, regarding the care and services identified in the plan of care.
Visit Narrative

Interventions/Instructions during visit:

- [x] Assess/Observe
- [x] Injection Administration
- [ ] Catheter change
- [ ] IV Therapy
- [x] Wound Care
- [x] Teaching/Instructions

Discharge Plans

Discharge patient to self care.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge to care of physician.
Able to stay in residence with primary caregiver's assistance or community agency's support.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge to caregiver.
Able to understand medication regime and care related to diagnoses.
Able to return to an independent level of care.
When goals are met.
Discharge when patient is independent in management of medical needs.
Discharge when goals met. Discharge when wound(s) healed.
Discharge when reliable caregiver available to assist with patient's medical needs.
When patient knows when to notify physician.

- [ ] Written notice of discharge provided to patient

Discharge scheduled for:
mm/dd/yyyy

Rehabilitation Potential

- ( ) Rehabilitation potential is poor
- (x) Rehabilitation potential is fair
- ( ) Rehabilitation potential is good
- ( ) Rehabilitation potential is excellent

Others:

OTHER SUPPLEMENTALS

Endocrine

- [ ] WNL (Within Normal Limits)

Is patient diabetic?	(x) Yes	( ) No	
Insulin dependent?	(x) Yes	( ) No	**How Long?**
Is patient independently able to draw up correct dose of insulin?	( ) Yes	(x) No	( ) N/A
Is patient able to administer own insulin?	( ) Yes	(x) No	( ) N/A
Is patient taking oral hypoglycemic agent?	(x) Yes	( ) No	( ) N/A
Is patient independent with glucometer use?	( ) Yes	(x) No	( ) N/A
Is caregiver able to draw up and administer insulin?	(x) Yes	( ) No	( ) N/A
Is caregiver independent with glucometer use?	(x) Yes	( ) No	( ) N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	(x) Yes	( ) No	( ) N/A

Does patient have any of the following?

- [ ] Polyuria
- [ ] Polydipsia
- [ ] Polyphagia
- [ ] Neuropathy
- [ ] Radiculopathy
- [ ] Thyroid Problems

Comments

Past Medical History (Mark all that apply)

- [ ] CHF
- [ ] Cardiomyopathy
- [ ] Arrhythmia
- [ ] Chest Pain
- [ ] MI
- [ ] CAD
- [ ] HTN
- [x] PVD
- [ ] Murmur
- [ ] Cancer: (Specify Type) — In remission? ( ) Yes ( ) No
- [ ] Osteoathritis/DJD: (Specify site affected)
- [ ] Joint Replacement: (Specify joint)
- [ ] Rheumatoid Arthritis
- [x] Gait Problems
- [ ] Fractures
- [x] Falls
- [ ] CVA
- [ ] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [ ] Dizziness/Vertigo
- [ ] IBS
- [ ] Crohn's Disease
- [ ] Diverticulitis/Diverticulosis
- [ ] Constipation
- [ ] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems: (Specify)
- [ ] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse: (Specify)
- [ ] Alzheimer's
- [ ] Mental Disorder: (Specify)
- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [x] Other: (Specify) Surgical wound.
- [ ] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis
- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [x] Diabetes
- [ ] Thyroid Problems
- [ ] COPD
- [ ] Asthma
- [ ] Emphysema

- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma

- [ ] Urinary Incontinence
- [ ] Urinary Retention
- [ ] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis: (Specify)
- [ ] Infectious Disease: (Specify)
- [ ] Tobacco Dependence

Type	Amount	Length of Time Used

Past Surgical History
left partial 5th-ray amputation dated 02/13/2026

Most Recent Immunization

Pneumonia
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Flu
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Tetanus
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

TB
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

TB Exposure
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Hepatitis B
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Additional Immunizations

Immunization Name
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Immunization Name
- ○ Yes ○ No ● Unknown Date mm/dd/yyyy

Comments

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.) | Date mm/dd/yyyy | Healthcare Professional or Clinic Site

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.) | Date mm/dd/yyyy | Healthcare Professional or Clinic Site

Booster 1
Product Name/Manufacturer (Lot No.) | Date mm/dd/yyyy | Healthcare Professional or Clinic Site

Booster 2
Product Name/Manufacturer (Lot No.) | Date mm/dd/yyyy | Healthcare Professional or Clinic Site

Health Screening

Last Cholesterol Level mm/dd/yyyy
Last Mammogram mm/dd/yyyy

Does patient perform monthly self breast exam? ○ Yes ○ No
Last Pap Smear mm/dd/yyyy
Last PSA mm/dd/yyyy

Last Prostate Exam mm/dd/yyyy
Last Colonoscopy mm/dd/yyyy

Fire Assessment for Patient with Oxygen

- [x] Patient is not using oxygen

Does patient have No Smoking signs posted? ○ Yes ○ No
- Are the Patient and Caregiver educated regarding this? ○ Yes ○ No

Does patient or anyone in the home smoke with oxygen in use? ○ Yes ○ No
- Are the Patient and Caregiver educated regarding this? ○ Yes ○ No

Are smoke detectors present and working properly? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Does patient have a properly functioning fire extinguisher? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Are oxygen cylinders stored properly? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Are all electrical cords near oxygen intact and free for fraying? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Does patient have an evacuation plan in case of fire? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Does patient refrain from using petroleum products around oxygen? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**
Does patient only used water-based body and lip moisturizers? • **Are the Patient and Caregiver educated regarding this?**	◯ **Yes** ◯ **Yes**	◯ **No** ◯ **No**

Comments

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness. Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue. Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure. Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest. Advise the patient to get up slowly to offset the feeling of dizziness. Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate. Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Dehiscence of amputation stump
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote wound healing and prevent infection at the amputation site.
Interventions: Assigned Staff: SN
Monitor vital signs and signs of infection (e.g., redness, swelling, discharge).
Progress Toward Goals:
No Progress Yet
Coordinate with the physician for potential need for antibiotics if infection is suspected.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Provide emotional and psychosocial support to the patient and family.
Interventions: Assigned Staff: SN
Conduct regular mental health assessments to address emotional well-being.
Progress Toward Goals:
No Progress Yet
Encourage open communication between the patient and family about feelings and challenges.
Progress Toward Goals:
No Progress Yet
Develop a care plan that includes regular check-ins to monitor emotional health.
Progress Toward Goals:
No Progress Yet

Problem: Type 2 diabetes with diabetic peripheral angiopathy and gangrene
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve blood glucose control to prevent complications.
Interventions: Assigned Staff: SN
Educate the patient and family on blood glucose monitoring techniques and target levels.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up visits to monitor glucose levels and medication adherence.
Progress Toward Goals:
No Progress Yet
Provide medication counseling for diabetes medications to ensure proper usage.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote wound healing and prevent further complications.
Interventions: Assigned Staff: SN
Conduct regular wound assessments and appropriate dressing changes.
Progress Toward Goals:

No Progress Yet
Educate the patient and family on proper foot care and hygiene practices.
Progress Toward Goals:
No Progress Yet
Coordinate with a wound care specialist for advanced treatment options.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance family support to improve the patient's mental and emotional well-being.
Interventions: Assigned Staff: SN
Offer educational resources about diabetes management and complications for family members.
Progress Toward Goals:
No Progress Yet

Problem: Cellulitis of left toe
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce inflammation and infection in the left toe
Interventions: Assigned Staff: SN
Administer prescribed antibiotics as per physician's orders
Progress Toward Goals:
No Progress Yet
Conduct regular wound assessments to monitor healing
Progress Toward Goals:
No Progress Yet
Educate the patient and family on signs of worsening infection
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote patient and family understanding of cellulitis management
Interventions: Assigned Staff: SN
Provide education on proper foot hygiene and care
Progress Toward Goals:
No Progress Yet
Involve family members in care routines and wound management
Progress Toward Goals:
No Progress Yet
Schedule follow-up appointments to monitor recovery progress
Progress Toward Goals:
No Progress Yet

Problem: History of falling
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the frequency of falls by 50% within the next 3 months.
Interventions: Assigned Staff: SN
Conduct a comprehensive fall risk assessment to identify environmental hazards.
Progress Toward Goals:
No Progress Yet
Provide education to the patient and family on fall prevention strategies.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up visits to monitor progress and make necessary adjustments.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve the patient's confidence in mobility and independence.
Interventions: Assigned Staff: SN
Provide assistive devices (e.g., walkers, handrails) as needed.
Progress Toward Goals:
No Progress Yet

Problem: Long term (current) use of insulin
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain stable blood glucose levels within target range.
Interventions: Assigned Staff: SN
Educate the patient and family on proper insulin administration techniques.
Progress Toward Goals:
No Progress Yet
Monitor blood glucose levels regularly and adjust insulin dosage as needed.
Progress Toward Goals:
No Progress Yet
Set up a medication management plan to ensure adherence to insulin therapy.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Educate patient and family about signs and symptoms of hypoglycemia and hyperglycemia.
Interventions: Assigned Staff: SN
Conduct educational sessions for the patient and family on diabetes management.
Progress Toward Goals:
No Progress Yet
Create a reference guide for recognizing and responding to blood sugar fluctuations.
Progress Toward Goals:
No Progress Yet
Involve family members in training sessions to ensure they can assist with management.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote active engagement in the patient's self-management of diabetes.
Interventions: Assigned Staff: SN
Encourage the patient to track their blood glucose levels and insulin usage in a log.
Progress Toward Goals:
No Progress Yet
Assist the patient in setting personal goals for diabetes management.
Progress Toward Goals:
No Progress Yet
Review and adjust the care plan regularly to align with the patient's progress.
Progress Toward Goals:
No Progress Yet

Problem: Long term (current) use of oral hypoglycemic drugs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain optimal blood glucose levels within target range.
Interventions: Assigned Staff: SN
Educate the patient and family about the importance of regular blood glucose monitoring and how to use glucose monitoring devices.

Progress Toward Goals:
No Progress Yet
Develop a medication schedule to ensure adherence to oral hypoglycemic therapy.
Progress Toward Goals:
No Progress Yet
Monitor the patient's blood glucose levels regularly and adjust medication dosages as prescribed by the physician.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance knowledge about diabetes management among the patient and family.
Interventions: Assigned Staff: SN
Create a written plan for diabetes management that includes dietary, medication, and exercise components.
Progress Toward Goals:
No Progress Yet
Provide resources and handouts on diabetes management techniques to the patient and family.
Progress Toward Goals:
No Progress Yet
Set up a follow-up schedule to review the patient's understanding and adherence to the diabetes management plan.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Prevent complications related to long-term use of oral hypoglycemic drugs.
Interventions: Assigned Staff: SN
Schedule regular foot assessments to prevent diabetic foot ulcers.
Progress Toward Goals:
No Progress Yet
Encourage routine eye examinations to monitor for diabetic retinopathy.
Progress Toward Goals:
No Progress Yet
Ensure regular health check-ups to monitor kidney function and other potential complications.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the patient's risk of falls at home.
Interventions: Assigned Staff: SN
Conduct a home safety assessment to identify and mitigate fall hazards.
Progress Toward Goals:
No Progress Yet
Review and optimize medications to minimize side effects that may contribute to falls.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance the patient's mobility and confidence in movement.
Interventions: Assigned Staff: SN
Provide education to the patient and family on proper techniques for safe ambulation.
Progress Toward Goals:
No Progress Yet
Monitor and document the patient's progress in mobility and balance.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-ups to assess mobility improvements and adjust the care plan as necessary.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving measures.
Interventions: Assigned Staff: SN
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores. Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (03/26/2026):

Assigned Staff: Garcia Alfonso, Kelvyn RN

Electronically Signed by Garcia Alfonso, Kelvyn RN 03/26/2026